

NO ONE TAUGHT YOU HOW TO BE HELD. SO YOU LEARNED NOT TO NEED IT.

REACTIVE ATTACHMENT

◇ PROTOCOL · THE ADULT AFTERMATH OF EARLY ATTACHMENT DISRUPTION

There is no adult diagnosis called Reactive Attachment Disorder. RAD is a childhood-only classification, requiring documented evidence of grossly insufficient early care. Most self-help content quietly glosses over that distinction. This one won't.

What actually persists into adulthood isn't the diagnosis. It's the template -- the reflex to distrust comfort, to leave before you're left, to read closeness as threat instead of relief. This protocol works with that template directly, not with a label you were never formally given.

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- EVIDENCE VS. STORY
 - BODY SCAN
 - MIRROR CHECK + FUTURE ECHO
 - COST LEDGER
 - LIE DETECTOR
 - CO-REGULATION GROUNDING

THE REASON BEHIND THIS PROTOCOL

THE DIAGNOSIS ENDS AT CHILDHOOD. THE TEMPLATE DOESN'T.

The DSM-5 classifies Reactive Attachment Disorder as a trauma- and stressor-related condition of early childhood, caused by social neglect or grossly insufficient caregiving. It's typically identifiable before age five. Affected children struggle to seek or accept comfort, show blunted positive emotion, and often live in a fight, flight, or freeze pattern well before they have language for any of it. There is no equivalent adult diagnosis.

That doesn't mean it ends there. A community-sample study following children diagnosed with RAD into adulthood found high rates of adult psychiatric diagnoses, substance use, and self-harm risk, well beyond what's explained by co-occurring conditions like ADHD. Early attachment disruption sets a template, and templates are what people carry, long after the diagnosis that named them stops applying.

This isn't a life sentence. It's a template, built once, under conditions that no longer apply. Templates can be rebuilt -- slower than you'd like, but rebuilt, through repetition, not through realization alone.

This work can surface old material fast, and it's frequently tied to real risk. If it feels unmanageable, or you're having thoughts of harming yourself, contact a crisis line (988 in the US, or your local equivalent) or a licensed trauma-informed therapist. That's not failure. It's the intervention this material was never meant to replace.

01

EVIDENCE VS. STORY · THE ATTACHMENT LOG

WHAT HAPPENED, AND WHAT DID YOU DECIDE IT MEANT?

Template beliefs about closeness usually trace to specific, nameable moments.

What is a specific memory of needing comfort and not receiving it, or receiving it unpredictably?

The memory is...

What did you decide about needing others, based on that?

I decided that needing people means...

02

BODY SCAN · THE CLOSENESS ALARM

WHAT DOES YOUR BODY DO WHEN SOMEONE GETS CLOSE?

For many adults with this history, real closeness triggers a threat response, not comfort.

What happens in your body when someone offers real closeness -- comfort, affection, dependency?

My body does...

What's the first urge -- pull away, test them, go numb, over-give?

My first urge is to...

03

MIRROR CHECK + FUTURE ECHO · WHO WROTE THE TEMPLATE

**WHO TAUGHT YOU CLOSENESS
WASN'T SAFE?**

Name the actual source of the template, then decide who runs your relationships now.

Whose caregiving taught you that needs go unmet, or get met unpredictably?

The caregiving that taught me this was...

In your closest relationship today, what would change if you responded to the present person instead of the template?

If I responded to who's actually here, I would...

04

COST LEDGER · WHAT SELF-RELIANCE COST

**WHAT HAS NEVER NEEDING ANYONE
COST YOU?**

The defenses that protected you as a child now charge rent in adulthood.

What has extreme self-reliance or emotional distance cost
you in relationships?

It has cost me...

What would you gain if you let one person in slightly
further than feels safe?

If I let someone closer, I might gain...

05

LIE DETECTOR · THE PERFORMANCE OF FINE

**WHAT DO YOU SAY INSTEAD OF "I
NEED YOU"?**

Name the substitute language you use instead of the need itself.

What do you say or do instead of asking directly for
comfort or reassurance?

Instead of asking, I...

What would the direct, honest version of the request
sound like?

The honest version would be...

SAFETY IS A SKILL YOU CAN PRACTICE IN SMALL DOSES.

GROUND YOURSELF BEFORE CLOSENESS, AND WHEN THE OLD ALARM FIRES.

TITRATE THE DOSE

BEFORE YOU LET SOMEONE CLOSE

You don't have to go from guarded to fully open in one step. Practice small.

- 01 Choose one low-stakes way to accept help or comfort this week -- let someone carry something, accept a compliment without deflecting it.
- 02 Name silently what you're feeling as it happens: "This is safety, not danger, even though it feels unfamiliar."
- 03 Give yourself permission to step back afterward if it's too much. Retreat is allowed -- it just isn't the only option anymore.

WHEN THE ALARM GOES OFF

RETURN TO THE PRESENT

Use this when closeness triggers the old alarm, not the current situation.

- 01 Name one true fact about the present moment: who is actually in the room, what year it is.
- 02 Place a hand somewhere on your own body and feel its weight. This is a body you're in now, not then.
- 03 Ask one honest internal question: "Is this person actually unsafe, or does this just feel like the old unsafe?"

THE WORK ISN'T DONE UNTIL IT LIVES IN YOUR ACTUAL LIFE.

06

INTEGRATION · CARRY IT FORWARD

WHAT DOES ONE SMALL ACT OF TRUST LOOK LIKE THIS WEEK?

Templates change through repetition, not through realization alone.

Name one specific person and one specific small way you'll let them closer this week.

This week, I will let ... closer by...

What will you tell yourself if it goes well, to help the new template stick?

If it goes well, I will tell myself...